



ASCEND

HEALTH INTELLIGENCE

ASCEND HEALTH INTELLIGENCE

AI-Augmented Sales & Marketing Platform

Built for Any Industry. Proven in Healthcare.

A Subsidiary of Certive Health Inc. · Scottsdale, Arizona

Investor
Presentation

2026



The Commercialization Gap

\$50B+

Spent annually on
healthcare innovation

<10%

Reaches meaningful
commercial scale

Every Industry

Faces the same
commercialization failure

Why great products fail to scale:

GTM Infrastructure Gap

Companies prove efficacy first — then scramble to build commercialization. By then, momentum is lost.

Fragmented Execution

Sales motions are siloed, inconsistent, and impossible to scale without costly headcount.

Unpredictable CAC

Customer acquisition costs are high, variable, and erode margins before products reach scale.

Industries don't have a product problem. They have a commercialization infrastructure problem. We solve it — and we've built our deepest expertise in healthcare.



Legacy Players Solve Pieces. Nobody Owns the Outcome.

CRM Platforms (Salesforce, Veeva)

→ Track activity — don't generate revenue

Data Providers (ZoomInfo, Definitive)

→ Static lists — no activation or execution

AI Point Tools

→ Insights without execution — no revenue accountability

Traditional Sales Teams

→ Expensive, inconsistent, tough to scale

No platform owns the full path from targeting → activation → revenue. Ascend does.

THE SOLUTION



ASCEND
HEALTH INTELLIGENCE

Ascend Health Intelligence

Any industry. Any market. One platform. Revenue outcomes — not just tools.

Intelligence Layer

AI identifies high-intent providers, partners, and patients via NPI-level targeting and behavioral signals.

Execution Layer

Multi-channel outreach, activation, and onboarding — fully managed and scalable without adding headcount.

Performance Layer

Revenue participation model — Ascend earns when clients earn. Aligned incentives, not software seats.

Built on 15+ years of commercialization execution • First deployment live: Ancillary Health signed and launching



Full-Stack Commercialization Engine

01

LAYER 1

Intelligence

AI identifies high-intent providers, partners & patients via NPI-level targeting and behavioral signals.

02

LAYER 2

Execution

Multi-channel outreach, activation, and onboarding — managed at scale, no headcount required.

03

LAYER 3

Conversion

CRM automation, pipeline tracking, and revenue performance dashboards with full attribution.

Learns from every interaction · Scales without adding headcount · Ascend earns when clients earn



Why Now

High-intent AI targeting is now viable across every industry — and healthcare is where the stakes are highest.

Provider margins at historic lows → forcing efficiency and outsourced commercialization

AI now enables compliant NPI-level targeting at scale — previously impossible

Data access has expanded across provider ecosystems — actionable signal is available now

Peptide & regenerative medicine markets opening under MAHA / RFK Jr. regulatory shift — timing is now

Legacy systems optimize workflows — Ascend is built to generate revenue

The market is shifting from tools → outcomes. Ascend is built for this shift — healthcare is where we prove it first.



Full-Stack Advantage

	Ascend	Salesforce	Veeva	Definitive
NPI-level provider targeting	Yes	Limited	Limited	Yes
Revenue participation model	Yes	No	No	No
Embedded commercialization execution	Yes	No	No	No
CRM + targeting + execution in one	Yes	Partial	Partial	No
Revenue generation vs workflow mgmt	Yes	No	No	No
Healthcare specialization	Yes	Partial	Yes	Yes

We don't just have data — we own the activation layer competitors don't operate in.



Why They Believe

“

Jeff Cline built 1-800-MEDIGAP into a 35,000-call-per-year machine. He knows how to turn healthcare data into predictable revenue better than anyone I've worked with.

Jeffrey Hayzlett — Former CMO, Eastman Kodak | C-Suite Network

“

Jeff Cline and his VRTCLS team led our go-to-market strategy and sharpened our positioning. That work was a key driver behind the momentum that supported our successful exit.

iStemCells Regenerative Medicine

“

Working with Jeff Cline transformed Elite Health Online from a manual founder-driven operation into a scalable digital services platform with materially improved efficiency.

Elite Health Online — Healthcare Digital Services

“

Their structured approach to market expansion directly supported measurable improvements in pipeline, conversion rates, and revenue growth.

Healthcare IT Services Client



Leadership

Jeff Cline

President / CMO

Built 1-800-MEDIGAP (35K calls/yr), iStemCells exit, 25+ years healthcare commercialization

Tom Marreel

CEO

Certive Health executive oversight, corporate governance, strategic partnerships

Tim Hyland

CFO

Healthcare finance leadership, financial systems, operational controls

Terry Newman

CTO / Security

HIPAA-compliant infrastructure, healthcare data security, regulatory compliance

STRATEGIC ADVISORY BOARD

Jeffrey Hayzlett

C-Suite Network | Former Fortune 100 CMO

Dr. Paul McHale

Healthcare Clinical Expertise

Dr. JJ Linder

Medical Advisor, Healthcare Innovation

Dr. Dan Graham

Clinical Advisor, Healthcare Systems



Ancillary Health Contract

CONTRACT SIGNED · Launches Mid-2026 · First Revenue Within 60–90 Days

NPI Targeting:	Pre-qualified physicians identified through NPI-level targeting and specialty segmentation.
Initial Programs:	Workers' comp pharmacy, post-surgical dressings, and wound care.
Revenue Ramp:	60–90 day implementation lead, then staggered cohort onboarding rather than batch activation.
Activation Plan:	56 physician activations Year 1 → 112 Year 2 → 282 Year 3.
Launch Ready:	Compliance documentation finalized, CRM workflows configured, outreach infrastructure activated.



Targeting High-Value Specialty Providers

\$3B

**Workers' Comp
Pharmacy Market**

Within \$37.6B total workers' comp medical spend

27.8K+

**Active U.S.
Orthopedic Surgeons**

Core target for specialty pharma expansion

\$6.1B

**U.S. Wound
Care Market**

10.5M Medicare beneficiaries with chronic wounds

\$11.1M+

**Peptide Year-1
Pipeline (AIM)**

Based on \$5K/\$10K/\$15K physician ramp model

Primary targets: pain management, orthopedics, psychiatry, and wound care — high-frequency, high-need specialty channels.

AIM peptide partnership unlocks a parallel physician network across regenerative medicine, longevity, and performance health.



3-Year Outlook — Ascend Health Intelligence Base Model

ASCEND HEALTH INTELLIGENCE - 3-Year Financial Forecast	Year 1	Year 2	Year 3
REVENUE	\$ 4,121,175	\$ 30,230,600	\$ 80,848,050
COST OF REVENUE	(1,294,500)	(1,692,000)	(1,836,000)
GROSS PROFIT	2,826,675	28,538,600	79,012,050
OPERATING & MANAGEMENT EXPENSES	(1,419,735)	(6,675,733)	(15,283,578)
NET INCOME BEFORE TAXES	1,406,940	21,862,867	63,728,472
INCOME TAX PROVISION	(364,397)	(5,662,483)	(16,505,674)
NET INCOME (LOSS)	\$ 1,042,543	\$ 16,200,384	\$ 47,222,798

Combined 3-year forecast across all Ascend Health Intelligence revenue streams: marketing services, health innovations, and AIM peptide revenue.



Base Model — Key Assumptions

Combined model across all Ascend Health Intelligence revenue streams: marketing services, health innovations, and AIM peptide revenue.

Revenue Assumptions

Marketing Platform — Yr 1: \$1.31M | Yr 2: \$2.99M | Yr 3: \$5.30M

Monthly platform fees (\$35K/\$36K/\$37K) plus success fees. Clients include Navi Nurses, RadNet, and Enrygy (in negotiation). Fee and revenue-share model.

Ancillary Health — Yr 1: \$1.04M | Yr 2: \$5.78M | Yr 3: \$10.75M

Success fees on physician program sales. 56 physicians activated Yr 1; 112 Yr 2; 282 Yr 3. Programs: workers' comp pharmacy, wound care, post-surgical dressings.

AIM Peptides — Yr 1: \$1.77M | Yr 2: \$21.46M | Yr 3: \$64.80M

AHI earns a 20% marketing fee on all AIM physician peptide revenue generated through the platform. Ramp: \$5K/mo (months 1–6), \$10K/mo (7–12), \$15K/mo (13+) per active physician.

Cost & Margin

Cost of Revenue

Yr 1: \$1.29M | Yr 2: \$1.69M | Yr 3: \$1.84M.
Covers physician outreach, activation labor, data licensing, and CRM infrastructure across all subsidiary entities.

Gross Margin

Yr 1: ~69% → Yr 2: ~94% → Yr 3: ~98%. Margin improves significantly as platform scales, particularly with high-margin Peptide revenue.

Operating Expenses

Yr 1: \$1.42M | Yr 2: \$6.68M | Yr 3: \$15.28M.
Includes management fees, AI/data platform, HIPAA compliance, intercompany charges, and sales enablement across all entities.

Tax Provision

~25.9% effective tax rate applied to net income before taxes each year.

Model Methodology

Bottom-Up Build

Revenue modeled client-by-client from signed and negotiated contracts. Not derived from top-down market share estimates.

Conservative Scope

All three revenue streams included. CM monthly fees + CI success fees + AIM peptide revenue. Intercompany charges: 15%/13.5%/12% of revenue; service vendor charge: 60% of platform fees.

Platform Economics

Cost base grows sub-linearly as clients scale. Marginal cost per physician activation decreases as infrastructure matures.

Key Risks

Remaining pipeline clients convert from negotiation; physician activation pace holds; no material regulatory disruption to core verticals.



Client Status — Q2–Q4 2026

Only Ancillary Health is signed. All others are in proposal or negotiation stages.

Ancillary Health	SIGNED	Workers' comp pharmacy, wound care, post-surgical. Launches mid-2026. First revenue 60–90 days.
RadNet	IN NEGOTIATION	AZ/TX radiology imaging pilots (mammogram) → nationwide expansion if successful.
Navi Nurses	IN NEGOTIATION	Platform fee \$22.5K/month + market entry fees + 5–10% revenue share.
Enrgy	IN NEGOTIATION	Proprietary light-based vitamin D therapy commercialization.
AIM — Peptides	IN NEGOTIATION	Peptide medicine & regenerative programs via AIM (im-athletic.com). Included in base model forecast. See dedicated AIM slides for full model detail.



Investment Highlights

Signed Ancillary Health contract with launch underway — first revenue expected within 60–90 days of activation.

Forecast built on contracted initial deployment — physician activation drives recurring participation economics.

Strong contribution margin — platform economics vs. a linear services model.

AIM Peptide partnership included in base forecast: \$11.1M Year-1 program sales; AHI earns a 20% marketing fee on all revenue generated through the platform. Scales to \$405M program sales by Year 3.

Client pipeline (RadNet, Navi Nurses, Enyrgy) — diversified revenue and expansion optionality.

Long-term upside: AI-augmented commercialization across healthcare verticals and adjacent industries — no natural ceiling.



\$2M for 10% Equity — \$20M Valuation

\$2,000,000

10% Equity · \$20M Post-Money Valuation

WHY \$20M VALUATION

Revenue in 60–90 days

Ancillary Health activations begin immediately — validates the platform before Series A

\$30M–\$50M Series A

Post-Year-3 unit economics unlock the next round at 3–5x this valuation

Ground-floor pricing

Intentionally investor-friendly — this is the lowest this platform will ever be priced

Structure:

Priced equity round — 10% for \$2M (\$20M post-money valuation)

Reporting:

Quarterly financial updates with KPI dashboards

Board:

Investor observer rights

Timeline:

Round closes June 19th, 2026



The Peptide Opportunity — AIM Partnership & Federal Changes

\$164B

Global Peptide Market 2026

Grand View Research, 2026

\$295B

Projected Market Size by 2033

Grand View Research, 2026

+8.73%

CAGR 2026–2033

Grand View Research, 2026

WHAT IS AIM

Athletic Integrative Medicine (AIM) — im-athletic.com — is the exclusive sales and distribution partner for Project Y Biotech. AIM delivers physician-prescribed peptide therapies across high-demand clinical categories.

AIM PEPTIDE PORTFOLIO

- Weight Loss & Metabolism (GLP-1 adjacent)
- Cognitive & Neuro Health — ADHD, Alzheimer's prevention
- Regenerative Medicine & Wound Healing
- Anti-Inflammatory & Cell Health
- Longevity, Anti-Aging & Performance Optimization
- NAD+ & Biohacking Protocols

THE FEDERAL REGULATORY TAILWIND

RFK Jr. announced ~14 of 19 Category 2 peptides returning to legal compounding status - *HHS / Joe Rogan Experience, Feb 2026*

FDA scheduled PCAC advisory meeting July 23–24, 2026 to evaluate BPC-157, TB-500 and 5 more peptides for Section 503A compounding - *FDA Federal Register, Docket FDA-2025-N-6895, Apr 16 2026*

HHS directed removal of 12 peptides from Category 2 on Apr 15, 2026 — pending final PCAC review - *FDA 503A Bulk Drug Substances List, Apr 15 2026*

Google searches for "peptides" up 80% and "cost of peptide therapy" up 300% (Apr 2025–Apr 2026) - *Open Loop Health / Google Trends, 2026*

AIM + Ascend positioned BEFORE final reclassification — first-mover physician activation advantage

Ascend + AIM = the commercialization engine that captures physician-level peptide revenue before the market fully opens.



How Ascend + AIM Generate Revenue Together

Ascend provides the commercialization engine. AIM provides the peptide products and physician network. Revenue flows through a shared participation model.

01 IDENTIFY

Target Physicians

Ascend AI targets high-intent MDs via NPI-level data: pain management, longevity, regenerative, and functional medicine specialists.

20,000+ physicians
in addressable peptide network

02 ACTIVATE

Enroll & Onboard

Multi-channel outreach activates physicians into AIM's prescriber network. Ascend manages compliance, CRM workflows, and onboarding at scale.

10 physicians Month 1
→ 960 by end of Year 1

03 PRESCRIBE

Peptide Revenue Flows

Active physicians prescribe AIM peptide protocols. AIM fulfills through Project Y Biotech. Revenue tracked per physician per program.

\$5K–\$15K/physician/mo
3-tier revenue ramp (mos. 1–6/7–12/13+)

04 EARN

Ascend Participates

AHI earns a 20% marketing fee on all revenue generated through the platform. As physicians activate and prescribe, AHI's fee accrues automatically on every sale driven by the Ascend commercialization engine.

\$1.77M Year-1

Ascend earns when physicians activate. Ascend earns when physicians prescribe. Zero revenue until the model works.



AHI + AIM + Ancillary

Ancillary sourced the AIM opportunity. AHI is the commercialization engine powering both. Together they drive physician revenue at scale.

ANCILLARY HEALTH

Relationship Originator + Physician Partner
SIGNED

- ✓ Sourced and introduced the AIM opportunity to AHI
- ✓ Manages signed and enrolled physicians within the AIM program
- ✓ Physician network oversight and program pricing

CLINICAL EXPERTISE

Dr. Dan Graham

Clinical Advisor — physician protocol guidance and program oversight for all enrolled physicians

ACTIVE PROGRAMS

Workers' comp pharmacy · Wound care · Post-surgical dressings

Yr 1: \$1.04M → Yr 3: \$10.75M

AHI

The Commercialization Platform

- NPI-level physician targeting
- Multi-channel physician outreach & activation
- CRM onboarding & compliance management
- 20% marketing fee on all revenue generated through the platform

DELIVERED TO BOTH
ANCILLARY + AIM

LEADERSHIP

Jeff Cline · Tom Marreel · Tim Hyland

AHI earns when clients earn.
Zero revenue until activation.

AIM — PEPTIDES

Client — Commercialization Buyer
IN NEGOTIATION

PHYSICIAN ACCESS SOURCED VIA

- Ancillary Health — physician network management
- AHI — AI physician acquisition engine

WHAT AIM PROVIDES

- ✓ Peptide portfolio via Project Y Biotech
- ✓ \$5K–\$15K / physician / month revenue ramp

AHI REVENUE FROM AIM

\$1.77M Yr 1 → \$64.8M Yr 3

AHI earns a 20% marketing fee on all revenue generated through the platform



What AHI Delivers to Each Partner

AHI is the outsourced commercialization firm and sales channel for both Ancillary programs and the AIM physician network.

AHI → ANCILLARY HEALTH

Services AHI Provides

01 NPI Targeting

AI identifies high-intent physicians by specialty, prescribing history, and behavioral signals — pre-qualified before first contact.

02 Physician Outreach & Activation

Multi-channel outreach (email, direct mail, digital ads) drives physician enrollment into Ancillary programs.

03 CRM & Contract Management

Full CRM onboarding, contract tracking, and physician management — Ancillary focuses on oversight, AHI handles the infrastructure.

04 Revenue Tracking & Reporting

Success fees tracked and reported per physician activation across workers' comp, wound care, and post-surgical programs.

AHI → AIM PEPTIDES

Services AHI Provides

01 AI Physician Acquisition

Scans 20,000+ physician NPI records. Targets pain management, longevity, orthopedics, and functional medicine specialists.

02 Multi-Channel Outreach & Activation

Email, direct mail, digital ads, and peer referral campaigns enroll physicians into the AIM prescriber network.

03 Onboarding, Compliance & CRM

HIPAA-compliant CRM setup, prescribing protocol delivery, practice staff training — physicians are active on AIM within 30 days.

04 Revenue Participation Tracking

AHI earns a 20% marketing fee on all AIM peptide revenue generated through the platform — automatically accruing on every physician sale driven by the Ascend commercialization engine.

AHI earns when clients earn — zero revenue until physician activation.



Why Physicians Sign Up for the AIM Platform

Physicians gain a new revenue stream and access to cutting-edge therapies. AHI handles everything else — compliance, onboarding, CRM, and fulfillment support.

\$5K–\$15K

New Revenue / Month

Prescribing AIM peptides adds a high-margin recurring revenue stream with no capital investment or inventory risk.

Zero Lift

Turnkey Practice Integration

AHI handles onboarding, CRM setup, compliance documentation, and fulfillment logistics. Physicians prescribe — AIM and AHI handle the rest.

First Mover

Access to Emerging Therapies

Offer patients cutting-edge peptide protocols — weight loss, cognitive health, regenerative medicine, longevity, and NAD+ — before the broader market opens post-FDA reclassification.

Fully Supported

Compliance & Clinical Protocols

AHI provides HIPAA-compliant documentation, pre-built prescribing protocols, and ongoing clinical support. Physicians focus on patient care — not administrative overhead.



How AHI Targets, Acquires & Activates Physicians

AI-driven identification to AIM platform activation — a fully managed, scalable physician acquisition funnel that learns and improves with every cohort.

01 IDENTIFY

AI scans 20,000+ physician NPI records. Filters by specialty, prescribing history, and behavioral signals.

Targets:

Pain mgmt, longevity, orthopedics, functional medicine, psychiatry

02 REACH

Multi-channel outreach campaign: email, direct mail, digital ads, and peer referrals.

Message:

New revenue stream, proven protocols, zero admin overhead

03 QUALIFY

Discovery call, platform demo, compliance review, and practice fit assessment.

Filter:

License verification, DEA status, practice volume

04 ONBOARD

CRM activation, compliance docs signed, prescribing protocols delivered, practice staff trained.

Timeline:

Active on AIM platform within 30 days

05 EARN

Physician prescribes via AIM platform. Revenue flows to AIM → AHI earns a 20% marketing fee on all revenue generated through the platform.

Per physician:

\$5K → \$10K → \$15K/mo ramp

10 physicians Month 1 → 960 by end of Year 1 → 4,040 by end of Year 3

20,000+ total addressable physicians. Model uses 4,040 (20%) over 3 years — a conservative penetration rate.



3-Year Physician Activation & Revenue Trajectory

Ascend accumulates revenue on peptide sales. Ramps from 10 physicians (Month 1) to 4,040+ by end of Year 3.

960

Physicians by Year 1

\$11.1M

Year-1 Program Sales Volume

\$1.77M

Year-1 Ascend Share

\$405.0M

Year-3 Program Sales Volume

Year	Physicians	Program Sales	Ascend's Share
Year 1	960	\$11,050,000	\$1,768,000
Year 2	2,400	\$134,150,000	\$21,464,000
Year 3	4,040	\$405,000,000	\$64,800,000

Source: Peptides tab — Ascend NPI Forecast Model (Peptide Assumption)



AIM Peptide Model — Key Assumptions

Physician Activation

Starting Point

10 physicians activated in Month 1 via Ascend AI-driven NPI targeting. Target specialties: pain management, longevity, regenerative, and functional medicine.

Ramp Rate

Yr 1: 960 physicians | Yr 2: 2,400 | Yr 3: 4,040.
Monthly activation cadence assumes multi-channel outreach and CRM-managed onboarding.

Addressable Network

20,000+ physicians in the U.S. addressable peptide prescriber network. Model uses 4,040 (20%) over 3 years — a conservative penetration rate.

Activation Cost

Managed entirely within Ascend platform. No per-physician headcount added; activation scales via AI and automated outreach infrastructure.

Revenue

Per-Physician Revenue

\$5K/month (months 1–6), \$10K/month (months 7–12), \$15K/month (month 13+) per active physician. Derived from AIM peptide portfolio pricing across weight loss, neuro, regenerative, and longevity protocols.

Program Sales Volume

Yr 1: \$11.1M | Yr 2: \$134.2M | Yr 3: \$405.0M.
Calculated using per-physician ramp model applied across activation cohorts.

Ascend Revenue Share

Yr 1: \$1.77M | Yr 2: \$21.5M | Yr 3: \$64.8M.
Revenue recognized when physicians prescribe; zero revenue until activation occurs.

Model Methodology & Risks

Revenue Recognition

Revenue earned on confirmed physician prescriptions. Zero revenue recognized until a physician is activated and prescribes an AIM protocol.

Regulatory Tailwind

Model is supported by RFK Jr. / HHS announcement restoring ~14 peptides to legal compounding. FDA PCAC review scheduled July 2026. First-mover advantage assumed.

Upside Not Modeled

Model uses \$5k–\$15k ramp-up model physician assumption as addressable pool. Full NPI universe is significantly larger. Additional peptide SKUs and new protocols represent unmodeled upside.

Key Risks

FDA reclassification timing; physician adoption pace; AIM/Project Y supply chain; variance from physician ramp model (\$5K/\$10K/\$15K monthly tiers).

Source: Ascend NPI Forecast Model. This forecast reflects management's assumptions. Actual results may differ materially based on regulatory timing, physician adoption, and AIM product availability.



Ascend Health Intelligence — Peptides Only (3-Year Forecast)

AHI - PEPTIDES ONLY - 3-Year Financial Forecast	Year 1	Year 2	Year 3
REVENUE	\$ 1,768,000	\$ 21,464,000	\$ 64,800,000
COST OF REVENUE	(282,000)	(330,000)	(378,000)
GROSS PROFIT	1,486,000	21,134,000	64,422,000
OPERATING & MANAGEMENT EXPENSES	(583,400)	(4,396,840)	(11,634,000)
NET INCOME BEFORE TAXES	902,600	16,737,160	52,788,000
INCOME TAX PROVISION	(233,773)	(4,334,924)	(13,672,092)
NET INCOME (LOSS)	\$ 668,827	\$ 12,402,236	\$ 39,115,908

Revenue is based on marketing agreement and revenue share of AIM physician peptide sales. Physician ramp: \$5K/mo (months 1–6), \$10K/mo (months 7–12), \$15K/mo (month 13+).



\$2M — 1-Year Deployment Plan

45% \$900,000

Ancillary Health Physician Activation · Q1–Q2

Physician onboarding & activation campaigns · Targeted outreach infrastructure · Working capital for 60–90 day ramp

30% \$600,000

AI / Data Platform Foundation · Q2–Q3

AI model development & NPI data enhancement · CRM and automation infrastructure · Performance analytics

25% \$500,000

Healthcare Operations · Q3–Q4

Infrastructure for scale & compliance · HIPAA, legal, and regulatory framework · Sales enablement

Total: \$2,000,000 · 10% Equity · \$20M Post-Money · Closes July 1, 2026



The Path to \$100M+ ARR (Year 3–5 with Peptides)

VALIDATE

01

Prove the Model

~\$1M ARR

Ancillary Health and AIM peptide launch simultaneously validate the platform across two verticals.
Physician activation → recurring revenue → repeatable economics.

SCALE

02

Scale the First Clients

~\$60M ARR

At scale, 500+ active physicians per client supports ~\$60M ARR from a single large deployment, based on mature per-physician recurring economics.

REPLICATE

03

Replicate Across Industries

\$100M+ ARR

5 additional scaled clients across adjacent healthcare verticals and other industries adds \$40M+ ARR.
Same system, new markets.

One AI-augmented commercialization platform — deployable across healthcare and beyond — with repeatable targeting, activation, and revenue participation at every scale.